

Modernising Public Health Law in Tamil Nadu

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Executive Summary

Tamil Nadu has built one of India’s most effective public health systems through smart policies and strong administration. The state has controlled major diseases, reduced deaths in childbirth, and provided health insurance to millions. These successes show the state’s ability to deliver results. The next stage of evolution is to create a modern institutional framework by updating public health law to make proven practices into permanent frameworks. Modern public health legislation would provide authority for systematic disease prevention, health monitoring, and coordinated policy implementation that administrative action alone cannot achieve. This legal foundation would transform Tamil Nadu’s policy innovations into lasting institutional commitments, ensuring sustained health improvements and continued leadership in public health practice.

1 Proposal

Tamil Nadu’s public health legal framework, anchored by the Tamil Nadu Public Health Act, 1939 (‘TNPHA’), stands as one of India’s earliest comprehensive public health laws. While groundbreaking for its time in establishing key institutions like the Public Health Board and recognising health’s connection to sectors like water and sanitation, this pre-independence framework is no longer aligned with Tamil Nadu’s modern public health strategy.

The TNPHA should be revised in accordance with the Directorate of Public Health and Preventive Medicine’s current assessment of public health priorities. This can be done by ensuring that the statute

- gives the Government the full range of modern legal tools to achieve those priorities

- has sufficient flexibility to allow for issuing rules and regulations to address future challenges and public health emergencies, so that repeated amendments are not required in emergencies, such as the Tamil Nadu Public Health (Second Amendment) Act, 2020
- creates adequate rights and safeguards for citizens while giving the Government adequate powers to achieve its public health goals

Any amendment process should also map and address the directorate's current priorities and pain points in terms of the administration of the TNPHA.

2 Problem Statement

The pre-independence legislation faces three key limitations that hamper its effectiveness in the present-day context:

2.1 Outdated regulatory approach

The TNPHA relies on a penalty-based system targeting activities that no longer represent primary public health concerns: occupying houses with inadequate water supply, improper waste disposal in private spaces, library book use during illness, and failure to control rat populations. This approach reflects the public health priorities of the 1930s rather than evidence-based risk assessment.

Modern public health legislation employs a broader range of regulatory tools. These include public warnings, disclosure requirements, health surveillance systems, and liability frameworks. Effective enforcement uses graduated responses—improvement notices and capacity-building programs before escalating to compensation requirements, closure orders, and penalties.

2.2 Imbalanced authority structure

The legislation grants extensive discretionary powers to state authorities without corresponding safeguards or obligations. Contemporary public health law emphasizes balanced governance that establishes clear rights and obligations for both citizens and government. This includes mechanisms for informed consent, transparent decision-making processes, and citizen participation in health policy development.

2.3 Misalignment with current priorities

The framework focuses predominantly on infectious disease control while Tamil Nadu's health challenges have shifted significantly. Non-communicable diseases now account for 70% of deaths and 65% of disability-adjusted life

Table 1 Modern regulatory strategies and examples

This table outlines a typology of regulatory strategies commonly employed in public health law, ranging from traditional command-and-control to more flexible, market-oriented or participatory models. Each approach is illustrated with a real-world statutory example, demonstrating how legal mechanisms can vary across jurisdictions to influence public health outcomes.

Strategy	Statutory Example
Command-and-control	“No person who knows that he is suffering from a notified disease shall take any book, or cause any book to be taken for his use, or use any book taken, from any public or circulating library.” <i>Tamil Nadu Public Health Act, 1939, s 73</i>
Public disclosure mandates	Every licensed food premises must “display a copy of so much of the licence as is relevant to the premises” in a conspicuous place. <i>New South Wales Food Regulation 2015 (Australia) cl. 53(1)</i>
Choice architecture	Chain outlets must print the kilojoule content of each menu item and the statement “The average adult daily energy intake is 8700 kJ” on all menus, boards and labels, steering choices without banning anything. <i>New South Wales Food Regulation 2015 Part 5 cls. 33–35</i> (under Food Act 2003 s 106N)
Negotiated rulemaking	The Chief Health Officer must publish every draft public-health policy for public comment and invite submissions before recommending it to the Minister. <i>Western Australia Public Health Act 2016 s. 49</i>
Industry self-regulation	A licensee whose business operates under an <i>accredited food-safety program</i> (written by the business, audited by government) “must comply with the program” in daily operations. <i>Queensland Food Act 2006 s. 123</i>
Deregulation	“This Regulation shall not apply to the direct supply, by the producer, of small quantities of primary products to the final consumer or to local retail establishments directly supplying the final consumer.” <i>Regulation (EC) No 852/2004, Art. 1(2)(c)</i> .

The list of strategies in the left column is from Lawrence O Gostin, *Public Health Law: Power, Duty, Restraint* (2nd edn, University of California Press 2008) 208, fig 6.1.

years in the state. With diabetes affecting over 40% of households, the legal framework fails to address the state’s primary health challenges.

The disconnect is particularly evident when comparing the law’s focus with Tamil Nadu’s current health policies. Both the Tamil Nadu State Health Policy – Vision 2030 and the Tamil Nadu Health System Reform Program emphasize NCD prevention and management, yet the legal framework provides no foundation for these priorities.

3 Annexure 1 - Statement of relevant experience

3.1 Engagements with the government

1. Worked with the Government of India on drafting a new law for public health emergencies.
2. Proposed pathways to state governments such as Rajasthan to establish an implementable rights-based health framework, including concerns pertaining to public health.
3. Engaged by the Law Commission of India for the 256th Report on Eliminating Discrimination Against Persons Affected by Leprosy.
4. Recommended changes to laws that discriminate against persons affected by leprosy, through engagements with state leprosy officers, state disability commissioners, and the Union Ministry of Social Justice and Empowerment.
5. Worked with the Government of India on updating the Transplantation of Human Organs and Tissues Act, 1994.
6. Advised state governments of Odisha and Maharashtra to implement the Supreme Court’s guidelines for withholding/withdrawal of life-sustaining treatment by preparing a comprehensive draft framework of orders for state adoption.
7. Drafted the National Medical Commission Bill, 2014.
8. Prepared the National Health Data Management Policy and consequent notifications.

3.2 Original research

1. Conducted doctrinal and empirical research on central and state-level laws dealing with public health and the right to health, developing recommendations to make them modern, people-centric, evidence-based, and realisable.

Table 2 Evolving priorities in public health law

The Tamil Nadu Public Health Act, 1939 was pioneering in its time, but its design reflects the public health priorities and governance philosophy of a different era—one focused on policing hygiene and punishing individual non-compliance. As Tamil Nadu considers consolidating and modernising its public health legislation, it is essential to recognise how contemporary public health frameworks have evolved globally. The table below contrasts the approaches found in older statutes like the 1939 Act with those embedded in modern public health laws, particularly in relation to public health emergencies (PHEs), non-communicable diseases (NCDs), and coordinated, rights-respecting governance.

Priority	Early 20th century	Modern regulation
Infectious disease control	Punitive measures: fines for failure to report illness, compulsory isolation using police powers	Integrated response to public health emergencies: surveillance networks, digital contact tracing, tiered containment, human rights safeguards
Sanitation and hygiene	Personal and household cleanliness enforced by local inspectors	Systems thinking: solid waste management, sewerage, water quality, urban design to promote hygiene
Health governance	Centralised authority in local health officers Limited inter-departmental coordination	Multi-level governance: empowered health authorities, defined agency roles, coordination during PHEs
Health information systems	Passive reporting by doctors and citizens, often paper-based	Active surveillance: digital epidemiology, real-time dashboards, registries for NCDs and infectious diseases
Chronic disease and lifestyle risks (NCDs)	Largely unaddressed; viewed as individual moral failings	Population-level interventions: food environment regulation, tobacco/alcohol control, physical activity promotion
Environmental and zoonotic health	Treated as nuisance abatement (e.g. stagnant water, pests)	Integrated environmental health: air/water quality regulation, vector control, One Health for zoonotic risks
Community engagement	Citizens as subjects of compliance Health seen as a police matter	Risk communication, behavioural nudges, co-production of public health outcomes with communities
Legal philosophy	Command-and-control Penal approach to compliance	Evidence-based regulation, precautionary principle, proportionality and due process

2. Worked on legislative guidance on public health responses, especially the management of public health emergencies.
3. Instrumental in securing the Common Cause v. Union of India judgment at the Supreme Court in 2018 (and its subsequent modifications in 2023) which set guidelines for withholding/withdrawal of life-sustaining treatment.
4. Collaborated with doctors, hospitals, and patient groups to produce a toolkit on end-of-life care that guides stakeholders on their rights and responsibilities regarding withholding/withdrawing life-sustaining treatment, framing advance medical directives, and participating in advance care planning.
5. Created a model legal framework on end-of-life care in partnership with the End-of-Life Care in India Task Force to advance patients' rights and ethical best practices.
6. Drafted a background paper on decision-making for persons with impaired decision-making capacity, analysing the problem, examining gaps in the legal framework, and suggesting an approach to address this.
7. Published an exhaustive 3-part study of the accountability frameworks for healthcare providers in India – the court and consumer fora, regulation of clinical establishments, and regulation of medical practitioners.

4 Annexure 2 - Statement of general work experience

Since its inception in 2013, Vidhi has produced over 125 pieces of original research and worked with 15 Ministries in the Government of India, 4 State Governments, 8 public institutions, 3 High Courts, and the Supreme Court of India to convert its research into over 64 enactments of binding law or policy. Therefore, Vidhi has had significant experience in providing legal assistance to several ministries and public institutions.

An indicative list of Vidhi's clients as well as a brief description of some of its projects are provided below:

4.1 List of Clients

4.1.1 State and Local Governments

1. New Delhi Municipal Council
2. Municipal Corporation of Delhi

3. Karnataka Urban Infrastructure Development Financial Corporation
4. Rural Drinking Water and Sanitation Department, Government of Karnataka
5. The Education Department (Primary and Secondary Education), Government of Karnataka
6. Urban Development Department, Government of Karnataka
7. Directorate of Municipal Administration, Bengaluru, Government of Karnataka
8. Department of Land Revenue, Government of Nagaland
9. Chief Minister's Office, Government of Maharashtra
10. Delhi Commission for Protection of Child Rights, Government of National Capital Territory of New Delhi
11. Agriculture and Cooperation Department, Government of Telangana

4.1.2 Central Government Ministries and Departments

1. Ministry of Law and Justice
2. Department of Economic Affairs, Ministry of Finance
3. Department of Revenue, Ministry of Finance
4. Department of Financial Services, Ministry of Finance
5. Ministry of Home Affairs
6. Ministry of Health and Family Welfare
7. Ministry of Corporate Affairs
8. Department of Land Resources, Ministry of Rural Development
9. Department of Consumer Affairs, Ministry of Consumer Affairs, Food and Public Distribution
10. Ministry of Agriculture and Farmers Welfare
11. Department of Fertilisers, Ministry of Chemicals and Fertilisers
12. Ministry of Electronics and Information Technology
13. Delhi Cantonment Board (Ministry of Defence)
14. Department of Sports, Ministry of Youth Affairs and Sports
15. Ministry of Labour and Employment

16. Ministry of Commerce and Industry

4.1.3 High Courts and Lower Courts

1. The High Court of Gauhati
2. The High Court of Jammu and Kashmir
3. Mumbai City Civil and Sessions Court, Mumbai

4.1.4 Public Institutions and Regulatory Bodies

1. NITI Aayog
2. Food Safety and Standards Authority of India
3. Law Commission of India
4. Bureau of Police Research and Development
5. University Grants Commission
6. International Financial Services Centres Authority
7. Reserve Bank of India
8. Telecom Regulatory Authority of India
9. National Highways Authority of India
10. Unique Identification Authority of India
11. Insolvency and Bankruptcy Board of India
12. Karnataka Digital Economy Mission
13. National Health Authority

4.1.5 International Organisations

The World Bank Group

4.1.6 Finance Commissions

1. Fourteenth Finance Commission
2. Fifteenth Finance Commission

4.2 Brief description of certain key projects undertaken by Vidhi